

**ABI Statement of Best Practice
for Critical Illness Cover**

2009 Review Consultation Paper

Contents

	Page
Contents	2
1. Executive Summary	3
2. Proposals for Consultation	
▪ Background and brief history of Critical Illness (CI)	6
▪ The introduction of the Statement of Best Practice	6
▪ Market environment and the current review	6
▪ Total permanent disability (TPD)	7
▪ Resolving the issues	8
▪ Other options considered for TPD	9
▪ The range of model illness definitions	10
▪ Consultation questions	10
- Total permanent disability	10
- Improvements in clarity	11
- Children's critical illness pre-existing medical condition exclusion	12
▪ Transitional Arrangements	12
▪ Future work	13
3. Replying to this consultation paper	13
Appendix 1 – Proposed model definitions with headings	14
Appendix 2 – Other issues considered	18

1. Executive Summary

- 1.1 The Statement of Best Practice for Critical Illness Cover (the 'Statement') was first drafted in 1999 following an Office of Fair Trading (OFT) report on health insurance. The Statement introduced model definitions and wordings and allowed for a full review every three years, and interim reviews when needed, for example, due to changing regulations or legislation.
- 1.2 This consultation paper sets out our proposals for the 2009 review, which follows previous full reviews in 2006 and 2002 and an interim review in 2004. This review is an opportunity to ensure that the Statement continues to achieve its key aims of helping consumers in three important ways:
- **Security** – provides consumers with the safeguard that appropriate minimum standards of cover are used across the industry.
 - **Comparability** – makes it easier to compare critical illness (CI) insurance from different insurers.
 - **Clarity** – helps improve understanding about what each product does, and does not, cover.
- 1.3 In line with the Statement we are conducting this full review with the view that customers continue to get meaningful, understandable cover that meets their needs at an affordable price.
- 1.4 We are consulting with Insurers, Reinsurers, Distributors, Trade Bodies, Consumer Groups, Regulators and Medical Experts on the following changes to the Statement:
- 1.4.1 **Total permanent disability (TPD) within CI policies** – TPD is a condition now covered by most CI policies. There is no model definition for TPD so each insurer uses its own definition to assess whether someone has become totally and permanently disabled. These range from being permanently unable to return to the person's 'own' or 'any' occupation or unable to perform a defined number of specified tasks. Research shows that, while TPD claims account for a relatively low proportion of CI claims at around 3%, 55% of these are declined.

We are consulting on proposals for the Statement to stipulate that TPD, in any current form, should not be included within CI policies. We have devised a set of model definitions that if added to CI policies together aim to provide similar cover, but will be clearer to customers about what is and is not covered. In particular, an intention of this proposal is to make a clearer distinction between CI and Income Protection policies, by removing definitions from CI based on occupation. We also propose to introduce a new model definition for 'Loss of the physical ability to look after yourself'.

The proposed model definitions are contained in appendix 1 and comprise a combination of new model definitions (some of which are completely new and others are for conditions which some insurers already cover) and amendments to extend the scope of existing model definitions.

1.4.2 **Improvements in clarity** – we have identified three areas, from a number that had been raised, where being more explicit about the scope of cover would help consumers. In proposing the following three model definition changes, we do not consider that the scope of cover would change:

- **Terminal illness** – we are consulting on a change to the terminal illness definition to be clearer about when an illness is considered to be terminal.
- **Cutaneous lymphoma** – cutaneous (meaning ‘relating to the skin’) lymphomas are not covered under the current cancer definition as, like other non-melanoma skin cancers, they fall under the general skin cancer exclusion. Also in common with non-melanoma skin cancers, cutaneous lymphomas generally have a good prognosis. To avoid any future confusion, we propose to make it explicit in the model cancer definition that cutaneous lymphomas are not covered while they remain confined to the skin. If a cutaneous lymphoma were to spread beyond the skin, it would then be covered. Cover for malignant melanomas would also continue as before.
- **Parkinsonian syndromes** – these can sometimes be confused with Parkinson’s disease and are not currently covered. To avoid any potential future confusion, we propose to make it explicit in the Parkinson’s disease definition that Parkinsonian syndromes are not covered.

1.4.3 **Children’s Critical Illness pre-existing medical condition exclusion** – since the Statement first introduced model exclusions, the number of exclusions has reduced, resulting in less small print which allows consumers to compare policies more easily. We therefore feel it will help consumers to introduce a standard wording for the pre-existing medical condition exclusion that insurers apply to children’s critical illness cover.

Replying to this Consultation Paper

1.5 Views are invited on this consultation document. Please send your responses to arrive by 30 September 2009 to Karen Evans, Protection Policy Adviser.

- Post: Association of British Insurers, 51 Gresham Street, London EC2V 7HQ
- E-mail: health@abi.org.uk

1.6 The full list of questions for which we are inviting responses is as follows:

- Q1: Given the claims experience and number of declined claims of the current TPD definitions within CI, do you agree the industry is right to take action and address the issue?
- Q2: Do you agree with the approach to produce a set of definitions covering the underlying causes of TPD; if not do you have alternative proposals?
- Q3: Do you agree with the proposal to introduce a model definition for ‘Loss of the physical ability to look after yourself’?
- Q4: Do you have any suggested improvements to the proposed model definition wordings (see appendix 1)?
- Q5: Do you agree that the ABI Statement should stipulate that TPD, in any current form, should not be included within CI policies as part of best practice due to the

evidence base of consumer detriment and in view of competition dynamics?

- Q6: Do you support the intention to create clearer distinction between CI and IP benefits by preventing insurers including occupational based TPD definitions relating to the lump-sum CI benefit?
- Q7: What sales force education needs do you feel are required?
- Q8: What impact, if any, do you believe these changes to TPD would have on consumer demand for CI and IP products?
- Q9: Do you support the ABI's proposal to update the current model Terminal Illness definition in light of more effective treatment?
- Q10: Do you have any suggestions to improve the wording of the proposed model terminal illness definition in appendix 1?
- Q11: Do you support the proposal that the model cancer definition should be explicit that cutaneous lymphomas that remain confined to the skin are not covered; if not, do you have an alternative suggestion?
- Q12: Do you have any suggestions as to how this aspect of the model cancer definition wording (see appendix 1) could be improved?
- Q13: Do you support the proposal that the model Parkinson's disease definition should be explicit that Parkinsonian syndromes are not covered; if not, do you have an alternative suggestion?
- Q14: Do you have any suggestions as to how this aspect of the model Parkinson's disease definition wording (see appendix 1) could be improved?
- Q15: Do you support the ABI's recommendation to standardise the wording that insurers use for excluding pre-existing medical conditions for Children's Critical Illness cover?
- Q16: Do you have any suggestions to improve the wording of the model pre-existing medical conditions exclusion for children's critical illness cover in appendix 1?
- Q17: Are there any other points you would like to raise regarding the Statement review?
- Q18: Given the scope of the proposals in this consultation paper, do you believe that a twelve month implementation period is appropriate?

2. Proposals for consultation

Background and brief history of Critical Illness (CI)

- 2.1 CI insurance has been a very successful protection product. Initially there were many differences between the products that were available and this made them difficult to compare, particularly in the IFA sector. However the introduction of some model definitions for six illnesses in 1992, followed by generic terms and model wordings created greater consumer and IFA confidence in the product.
- 2.2 Over 2.8 million policies containing CI have been sold over the last five years. Even though CI insurance is a long-term contract where we expect most claims to be paid later as customers get older, we estimate that insurers have paid £730 million in critical illness claims for 2007 alone.

The introduction of the Statement of Best Practice

- 2.3 In 1998, the OFT published a report on health insurance. Amongst its conclusions, it said that there were too many different definitions between companies and this was causing consumer and adviser confusion. It called for the industry to reduce consumer detriment by adopting model illness definitions and a common format for product literature for CI. In response, in 1999 the Association of British Insurers (ABI), in consultation with OFT, published the Statement of Best Practice for Critical Illness Cover.
- 2.4 The Statement introduced model definitions and wordings. It allows for a full review every three years to ensure that it remains up to date (for example, with legislative and market changes and advances in medical science) and interim reviews as required, for example, by changing regulations. Any changes to existing wordings should be as a result of an industry-wide problem for customers and/or insurers and agreement that the proposed change will address the issue.
- 2.5 This consultation paper sets out our proposals for the 2009 review, which follows previous full reviews in 2006 and 2002 and an interim review in 2004. This review is an opportunity to ensure that the Statement continues to achieve its key aims of helping consumers in three important ways:
- **Security** – provides consumers with the safeguard that appropriate minimum standards of cover are used across the industry.
 - **Comparability** – makes it easier to compare CI insurance from different insurers.
 - **Clarity** – helps improve understanding about what each product does, and does not, cover.
- 2.6 To achieve these three objectives – of security, comparability and clarity – the Statement includes a common format for how insurers should present CI in product literature and model definitions for the illnesses most commonly covered. These are set at an “appropriate minimum standard” so that insurers have to use that definition, or may modify the definition in prescribed ways to offer at least the same cover.

Market environment and the current review

- 2.7 The insurance industry has done a huge amount to address reputational issues. The overwhelming majority of claims for CI are assessed, accepted and paid promptly. Our analysis shows that the very small proportion of claims that go wrong almost always do so for one of the following two reasons:
- **Non-disclosure** – this has been addressed by ABI guidance on clearer application forms and processes and the ABI Code on handling claims fairly.
 - **Disputes about what is covered** – previous work on this has been through the introduction of ABI model definitions and clearer, more descriptive headings.
- 2.8 The previous work on disputes about what is covered and on non-disclosure are already showing significant benefits for consumers. The number of declined claims referred to the FOS is reducing and consumers can take out CI with confidence. However, we are not complacent and this review looking primarily at ‘total permanent disability’ (TPD) builds on this previous work.

Total permanent disability

- 2.9 ‘Total permanent disability’ (TPD) is a condition now covered by most CI policies. It accounts for only 3% of industry CI claims and it therefore takes time for a clear picture of the claims experience to emerge. However, we have now built an evidence base that shows there are three issues around TPD within CI that this review aims to address. These are as follows:
- Although TPD accounts for only 3% of CI claims, 55% of these are declined.
 - TPD accounts for 35% of complaints to the Financial Ombudsman Service (FOS) about what is covered under CI.
 - Most CI claims are paid promptly whereas TPD claims often take a long time to assess causing hardship to customers through delays in receiving the policy benefits.
- 2.10 Although there is less evidence, our initial research shows that the issues around TPD are not confined to any specific type of definition used in the market, but span all current TPD definitions including those based on being permanently unable to perform your ‘own occupation’. Of the known complaints made to FOS in 2008 about claims concerning TPD definitions, 60% (12) were based on the person’s ‘own occupation’. Further evidence is welcomed on this issue to build as clear a picture as possible.
- 2.11 We would therefore judge the success of any solution by the reduction in the proportion of relevant declined claims arising from CI policies taken out after any measures have been adopted.
- 2.12 We believe that the current position is not sustainable. We also believe that only an industry led solution can address this issue effectively for the following reasons:
- To date, an effective market led solution has not emerged.
 - Standardisation has already been agreed by OFT and FSA for use in other complex areas as it helps to improve clarity – no one insurer can do this.

- There are significant barriers to change – e.g. quotation portals, preferred provider panels, underwriting changes etc.

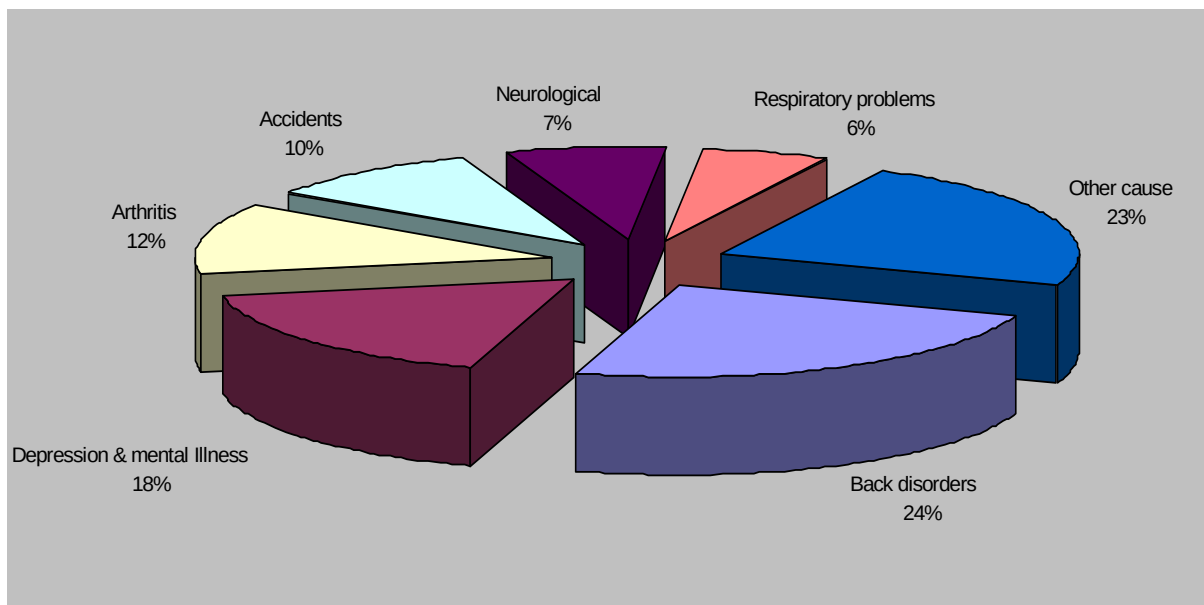
2.13 At present there is no model definition for TPD. Insurers use a wide range of definitions, from being permanently unable to return to their 'own' or 'any' occupation or unable to perform a defined number of tasks. Our evidence shows that there are many underlying medical and other reasons for the cause of the TPD claims which TPD aims to cover using a single definition. For example, the reasons can be as diverse as injuries caused by a car accident to a serious mental health condition. As a result, no single TPD definition is specific to the underlying reasons for all the diverse medical reasons for claims, and assessing claims is therefore often more subjective.

Resolving the issues

2.14 To address these problems an ABI working group was set up following discussion with the Office of Fair Trading, with a brief to address the issues around TPD whilst maintaining a similar level of cover and claims.

2.15 The group believed that much greater clarity could be achieved by analysing the past TPD claims and producing a set of specific model definitions with clear descriptive headings covering each of the main underlying medical reasons for the claims. It was felt that this would be more objective and clearer for both insurer and consumer. A further advantage would be that these definitions would not be linked to occupation, which can cause confusion with Income Protection. Accordingly, the group analysed around 1,200 past TPD claims to determine the underlying causes of claim.

2.16 The analysis showed that the underlying medical reasons for the claims were as shown in the chart below.



2.17 Replacing TPD with the set of definitions for the underlying medical reasons for the claims is not intended or expected to either increase or reduce premiums.

2.18 Many CI policies cover conditions that result in becoming unable to perform a number of specified tasks, sometimes as part of TPD and sometimes as a separate condition.

If our proposals for TPD are accepted, we expect that more insurers are likely to want to include this cover. Further, our consumer research showed that consumers clearly understood both the heading and the definition wording proposed for this purpose as set out in appendix 1. This proposed model definition is called 'Loss of the physical ability to look after yourself'.

- 2.19 An important consumer protection is that insurers cannot normally change the cover once a policy is in force. This means that these TPD changes would apply to new policies taken out after any changes in the new Statement are adopted by insurers.
- 2.20 The ABI working group notes that underwriting processes may need to change. Early discussions with underwriters have suggested that underwriting may be carried out with more precision compared to the current approach.
- 2.21 There are relatively very few standalone TPD policies in force. Accordingly, we have no industry-wide evidence available to show that the same issues apply. The proposals set out in this consultation paper are therefore confined to TPD included as part of a CI policy and do not apply to standalone or other TPD policies. Individual insurers will need to consider how they sell standalone TPD based on their own claims experience and their obligations to customers under TCF and our consumer research.
- 2.22 At the last Statement review the industry conducted consumer research into making headings much clearer. Similarly at this review we have carried out consumer research to check the clarity of the proposed new model definition headings. This ensures that consumers get an accurate impression of what is covered by each CI model definition at the point of sale.
- 2.23 Consumer research has shown that consumers don't understand the words 'total' and 'permanent' and if TPD is linked to occupation then it is confused with Income Protection (IP). This confusion was also found in consumer research conducted by the FSA¹. These proposals are intended to help draw a clearer distinction between CI and IP.

Other options considered for TPD

- 2.24 As part of the work to address the issues within TPD, the Protection Development Committee considered a number of possibilities.
- 2.25 The Statement already defines the term 'permanent'. A possibility was therefore also to define 'total disability'. However, this would not address the subjectivity in assessing TPD claims and would therefore be unlikely to solve the issues. Further, a range of different model TPD definitions would be needed to replicate the cover available in the market which would be confusing for consumers.
- 2.26 However, other areas for consideration might be:
 - **Clearer literature** – if a common approach can be developed for TPD, this may pave the way for recommendations for disclosure documents would be clearer about the scope of cover.

¹ See page 4 'payment expectations' at <http://www.fsa.gov.uk/pubs/consumer-research/crpr77.pdf>

- **Better training** – firms of advisers will need to ensure that staff are appropriately trained. If a common approach can be developed for TPD, this would be a more straightforward task given that this would be common across all insurers. Insurers would also be looking to communicate any changes to intermediaries.

The range of model illness definitions

- 2.27 Since the introduction of the Statement the principle used to determine whether a model definition is required is if a condition features on 75% of policies in the market then there should be a model definition available.
- 2.28 We would like to thank Defaqto for conducting a survey on behalf of the Protection Development Committee into the cover offered by individual CI insurers in the UK. The Defaqto report shows that since the last Statement was published in 2006, no new critical illness conditions have met the trigger threshold. We are pleased to note that the number of exclusions used appears to be falling. Consequently, we do not propose to introduce any new model critical illness definitions at this review, other than needed to support our consultation on TPD.
- 2.29 As the definitions of critical illnesses in most policies cannot be changed after the start of the policy it is important that the definitions used are appropriate for the full term, which may run for many years into the future. Our overriding aim is to enable consumers to continue to access products that are affordable and offer meaningful and understandable cover. We also recognise that insurers can only provide CI insurance if the premiums are expected to cover at least the amount they are likely to pay out plus the cost of providing appropriate financial reserves. The amount paid out in claims depends largely on what illnesses are covered and, crucially, insurers need to be able to estimate the likely amount paid out in the future to be able to set premiums.

Consultation questions

- 2.30 We are consulting with Insurers, Reinsurers, Distributors, Trade Bodies, Consumer Groups, Regulators and medical experts on the following changes to the Statement:
- 2.30.1 **Total permanent disability within CI** – we are consulting on proposals for the Statement to stipulate that TPD, in any current form, should not be included within CI policies. We have devised a set of model definitions that if added to CI policies together aim to provide similar cover, but will be clearer to customers about what is and is not covered. In particular, an intention of this proposal is to make a clearer distinction between CI and Income Protection policies, by removing definitions from CI based on occupation. We also propose to introduce a new model definition for ‘Loss of the physical ability to look after yourself’. The proposed model definitions are contained in appendix 1 and comprise a combination of new model definitions (some of which are completely new and others are for conditions which some insurers already cover) and amendments to extend the scope of existing model definitions.

Q1: Given the claims experience and number of declined claims of the current TPD definitions within CI, do you agree the industry is right to take action and address the issue?

Q2:	Do you agree with the approach to produce a set of definitions covering the underlying causes of TPD; if not do you have alternative proposals?
Q3:	Do you agree with the proposal to introduce a model definition for ‘Loss of the physical ability to look after yourself’?
Q4:	Do you have any suggested improvements to the proposed model definition wordings (see appendix 1)?
Q5:	Do you agree that the ABI Statement should stipulate that TPD, in any current form, should not be included within CI policies as part of best practice due to the evidence base of consumer detriment and in view of competition dynamics?
Q6:	Do you support the intention to create clearer distinction between CI and IP benefits by preventing insurers including occupational based TPD definitions relating to the lump-sum CI benefit?
Q7:	What sales force education needs do you feel are required?
Q8:	What impact, if any, do you believe these changes to TPD would have on consumer demand for CI and IP products?

- **Improvements in clarity** – we have identified three areas where being more explicit about the scope of cover would help consumers. In proposing the following three definition changes, we do not consider that the scope of cover would change:
 - **Terminal illness** – we are consulting on a change to the terminal illness definition to be clearer about when an illness is considered to be terminal.

Q9:	Do you support the ABI’s proposal to update the current model Terminal Illness definition in light of more effective treatment?
Q10:	Do you have any suggestions to improve the wording of the proposed model terminal illness definition in appendix 1?

- **Cutaneous lymphoma** – cutaneous (meaning ‘relating to the skin’) lymphomas are not covered under the current cancer definition as, like other non-melanoma skin cancers, they fall under the general skin cancer exclusion. Also in common with non-melanoma skin cancers, cutaneous lymphomas generally have a good prognosis.

To avoid any future confusion, we propose to make it explicit in the model cancer definition that cutaneous lymphomas are not covered while they remain confined to the skin. If a cutaneous lymphoma were to spread beyond the skin, it would then be covered. Cover for malignant melanomas would continue as before.

Q11:	Do you support the proposal that the model cancer definition should be explicit that cutaneous lymphomas that remain confined to the skin are not covered; if not, do you have an alternative suggestion?
Q12:	Do you have any suggestions as to how this aspect of the model cancer definition wording (see appendix 1) could be improved?

- **Parkinsonian syndromes** – these can sometimes be confused with Parkinson's disease and are not currently covered. To avoid any potential future confusion, we propose to make it explicit in the Parkinson's disease definition that Parkinsonian syndromes are not covered for the minimum standard definition.

Q13: Do you support the proposal that the model Parkinson's disease definition should be explicit that Parkinsonian syndromes are not covered; if not, do you have an alternative suggestion?

Q14: Do you have any suggestions as to how this aspect of the model Parkinson's disease definition wording (see appendix 1) could be improved?

- **Children's Critical Illness exclusion** – since the Statement first introduced model exclusions, the number of exclusions has reduced, resulting in less small print which allows consumers to compare policies more easily.

All insurers offering children's critical illness cover use a pre-existing medical conditions exclusion as an alternative to underwriting the children covered. This is needed for a variety of reasons, not least because some will be born after the policy starts. As more than 70% of CI policies in the market include this it has been suggested that this policy term should be standardised.

Q15: Do you support the ABI's recommendation to standardise the wording that insurers use for excluding pre-existing medical conditions for Children's Critical Illness cover?

Q16: Do you have any suggestions to improve the wording of the model pre-existing medical conditions exclusion for children's critical illness cover in appendix 1?

2.31 All the proposed definitions are included in appendix 1 of this document.

2.32 The proposals set out in this consultation paper have been discussed with the OFT.

Q17: Are there any other points you would like to raise regarding the Statement review?

Transition Arrangements

2.33 Following the consultation and taking into account the feedback provided, we hope to be in a position to publish the revised Statement of Best Practice for Critical Illness Cover by the year end. At that time we will also publish a transition timetable to allow insurers to review and amend their policies and literature for new business as appropriate. The final timetable will depend on the changes insurers will need to make. Historically we have allowed insurers twelve months to make any changes but would encourage changes to be adopted as soon as possible. Given the nature of the changes, we are consulting on whether twelve months to implement the proposals would be appropriate.

Q18: Given the scope of the proposals in this consultation paper, do you believe that a twelve month implementation period is appropriate?

Future work

- 2.34 The CI Consumer Guide and the CI Definitions Guide will be updated to reflect any changes.

3. Replying to this Consultation Paper

- 3.1 A co-ordinated response from each firm is invited on this consultation document. Please send your responses to arrive by 30 September 2009 to Karen Evans, Protection Policy Adviser.

- Post: Association of British Insurers, 51 Gresham Street, London EC2V 7HQ
- E-mail: health@abi.org.uk

Appendix 1 – Proposed model definitions with headings

New model definitions to replace TPD on CI policies

Cardiomyopathy – *resulting in a marked loss of the ability to do physical activity*

A definite diagnosis of cardiomyopathy by a Consultant Cardiologist. There must be clinical impairment of heart function for at least 3 months resulting in the loss of ability to perform physical activities to at least Class 3 of the New York Heart Association classification of functional capacity (marked limitation of physical activities where less than ordinary activity causes fatigue, palpitation, breathlessness or chest pain). The diagnosis must be supported by echocardiogram.

For the above definition, the following are not covered:

- All other forms of heart disease, heart enlargement and myocarditis.
- Cardiomyopathy secondary to alcohol or drug abuse.

Complex regional pain syndrome (CRPS) – *with clinical symptoms lasting for at least 12 months*

A definite diagnosis of complex regional pain syndrome (CRPS) in one or more limbs by the treating Consultant. The condition must have persisted for at least 12 months with the following clinical symptoms:

- Continuous intense pain in all or part of a limb;
- Chronic burning pain or allodynia (a painful response to a stimulus that does not usually cause pain); and
- Tissue changes – for example, redness of skin, swelling of joints, atrophy of the distal skin, tendency for the limb to be held stiffly.

For the above definition, the following are not covered:

- Other chronic pain or pain syndromes;
- Whiplash.

Chronic severe rheumatoid arthritis – *resulting in a loss of the ability to do specified physical activities*

A definite diagnosis by a consultant rheumatologist of chronic rheumatoid arthritis as evidenced by widespread joint destruction with major clinical deformity.

In addition the claimant must permanently satisfy three of the four following criteria:

Bending – The inability to bend or kneel to pick up something from the floor and stand up again and the inability to get into and out of a standard saloon car.

Dexterity – The inability to use hands and fingers to pick up and manipulate small objects such as cutlery, including being unable to write using a pen or pencil.

Lifting – The inability to lift, carry or otherwise move everyday objects by hand. Everyday objects include a kettle of water, a bag of shopping and an overnight bag or briefcase.

Mobility – The inability to walk a distance of 200 metres on flat ground, even with the aid of a walking stick if prescribed by a treating practitioner, and without having to rest.

Loss of a hand or a foot – *permanent physical severance*

Permanent physical severance of a hand or foot at or above the wrist or ankle joint.

Please note: This model definition would replace the current model definition for 'loss of hands or feet'

Respiratory failure – resulting in breathlessness even when resting

Advanced stage chronic lung disease resulting in:

- Breathlessness at rest;
- The need for continuous daily oxygen treatment ($\text{PaO}_2 < 7.3\text{kPa}$ when clinically stable as prescribed under British Thoracic Society and NICE guidelines) for at least 12 months.

Severe brain damage – resulting in permanent symptoms

A definite diagnosis by a Consultant Neurologist of death of brain tissue resulting in permanent neurological deficit with persisting clinical symptoms.

For this definition, the following are not accepted as evidence of permanent neurological deficit:

- Any symptoms of psychological or psychiatric origin;
- Fatigue;
- Headaches;
- Lethargy;
- Seizures.

Please note: This model definition would replace the current model definition for 'traumatic head injury'

Severe Crohn's disease – with symptoms that have not responded to optimal therapy

A definite diagnosis by a Consultant Gastroenterologist of Crohn's disease with symptoms that have not responded to optimal therapy while under the supervision of the Consultant.

There must also be evidence of continued inflammation with all of the following:

- Stricture formation causing intestinal obstruction requiring admission to hospital;
- Fistula formation between loops of bowel or bowel to another organ;
- At least one resection of a segment of small bowel.

Severe specified mental health conditions – causing an in-patient stay of at least 8 consecutive nights

A definite diagnosis by a Consultant Psychiatrist of any of the following conditions where the condition has required and resulted in an in-patient admission to a psychiatric in-patient ward for at least 8 consecutive nights;

- Bipolar affective disorder;
- Paranoid (delusional) psychosis;
- Schizo-affective disorder;
- Schizophrenia;

or

a definite diagnosis by a Consultant Psychiatrist of a severe depressive illness which has:

- chronic unremitting symptoms;
- not responded to comprehensive management and treatment for which the individual has complied, based on the best current UK clinical practice for more than 12 months; and
- required and resulted in an in-patient admission to a psychiatric in-patient ward for at least 8 consecutive nights.

For the above definition, the following are not covered:

- Conditions related to or exacerbated by alcohol or drug abuse.
- Other mental health problems such as mild or moderate depression, anxiety and stress.

Specific surgery for severe back and neck conditions

The undergoing of any of the following surgical procedures on the back or neck:

- Spinal fusion with instrumentation (excluding the cervical spine); or
- Stabilisation for a vertebral column fracture.

For the above definition, the following are not covered:

- Discectomy (the surgical removal of a disc);
- Spinal injections.

Proposed new model definition based on losing the ability to do three of six ADLs

Loss of the physical ability to look after yourself

Losing the physical ability to do at least three of the six tasks listed below with no reasonable prospect of there ever being any improvement. The person must need the help or supervision of another person and be unable to perform the task on their own, even with the use of special equipment routinely available to help.

1. **Washing** – the ability to wash in the bath or shower (including getting into and out of the bath or shower) or wash satisfactorily by other means.
2. **Getting dressed and undressed** – the ability to put on, take off, secure and unfasten all garments and, if needed, any braces, artificial limbs or other surgical appliances.
3. **Feeding yourself** – the ability to feed yourself when food has been prepared and made available.
4. **Maintaining personal hygiene** – the ability to maintain a satisfactory level of personal hygiene by using the toilet or otherwise managing bowel and bladder function.
5. **Getting between rooms** – the ability to get from room to room on a level floor.
6. **Getting in and out of bed** – the ability to get out of bed into an upright chair or wheelchair and back again.

Proposed revised model definitions to improve clarity
Cancer – excluding less advanced cases Any malignant tumour positively diagnosed with histological confirmation and characterised by the uncontrolled growth of malignant cells and invasion of tissue. The term malignant tumour includes leukaemia, lymphoma and sarcoma. For the above definition, the following are not covered: • All cancers which are histologically classified as any of the following: • pre-malignant; • non-invasive; • cancer in situ; • having borderline malignancy; or • having low malignant potential; • All tumours of the prostate unless histologically classified as having a Gleason score greater than 6 or having progressed to at least clinical TNM classification T2N0M0. • Chronic lymphocytic leukaemia unless histologically classified as having progressed to at least Binet Stage A. • Any skin cancer (including cutaneous lymphoma) other than malignant melanoma that has been histologically classified as having caused invasion beyond the epidermis (outer layer of skin).
Parkinson's disease [before age x] – resulting in permanent symptoms A definite diagnosis of Parkinson's disease [before age x] by a Consultant Neurologist. There must be permanent clinical impairment of motor function with associated tremor, muscle rigidity and postural instability. For the above definition, the following are not covered: • Parkinson's disease secondary to drug abuse • Parkinsonian syndromes
Terminal illness A definite diagnosis by the attending Consultant of an illness that satisfies both of the following: • The illness either has no known cure or has progressed to the point where it cannot be cured; and • In the opinion of the attending Consultant, the illness is expected to lead to death within [<i>the earlier of</i>] 12 months [<i>and the remaining term of the cover</i>].
Proposed model wording for Children's Critical Illness pre-existing medical conditions exclusion
[A claim will not be covered for children's critical illness cover if:] • The child's condition was present at birth; • The symptoms first arose before the child was covered; or • The child dies within [28 days] of meeting our definition of the critical illness.

Appendix 2 – Other issues considered

As part of the review, the position regarding the status of Polycythaemia rubra vera (PRV), Essential thrombocytopenia (ET) and other haematological malignancies was considered.

PRV and ET were reviewed in 2006 as part of the previous Statement process. Both conditions were confirmed as malignant and a diagnosis of either condition should be regarded as a valid claim under the cancer definition. We have seen no new evidence regarding these conditions and therefore propose that these conditions should remain covered under the model definition of cancer – excluding less advanced cases.

Other haematological malignancies, particularly myodysplastic syndromes (MDS), are more complex as they are often referred to as 'preleukaemia' which can lead to confusion and inconsistency in the market place in relation to claims payments. We do not propose to make any changes at this time but we will monitor these cases until the next Statement review in 3 years time.